Clinical Referral Coordination Exchange

Cedar Valley Care Network I Business and technical requirements I Draft 0.8

DOCUMENT PURPOSE AND SYSTEM CONTEXT

Cedar Valley Care Network is evaluating a referral coordination service connecting primary care clinics, specialty practices, and a central scheduling team. The current process relies on fax, phone calls, and manual status checks. This draft baseline captures business needs for discovery and vendor evaluation; it is not an approved clinical specification.

BUSINESS OWNER	**Ambulatory Services**
TECHNICAL OWNER	**Digital Health Integration**
TARGET USERS	**Referring clinicians, schedulers, specialty clinics, patients**
SOURCE SYSTEMS	**Two EHR platforms; vendor-specific APIs not yet confirmed**
DOCUMENT STATUS	**Draft for review - assumptions and open decisions remain**

FUNCTIONAL REQUIREMENTS

GOAL-001 PATIENT ACCESS

Reduce avoidable referral-to-appointment delay while keeping clinicians informed when a referral is incomplete, redirected, or declined.

FR-001 REFERRAL INTAKE

Accept electronic referrals containing patient identity, referral reason, urgency, requested specialty, relevant notes, and supporting documents. Preserve the source-system referral identifier.

FR-002 COMPLETENESS CHECKS

Check required fields and attachments for the selected specialty. Return incomplete referrals to the sender with a specific list of missing information.

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WORKFLOW REQUIREMENTS

FR-003 **TRIAGE AND ROUTING**

Allow authorized clinical staff to review referrals, assign urgency, route to an eligible specialty queue, and record a reason when changing the requested destination.

FR-004 **STATUS COMMUNICATION**

Show the referring clinic a current referral status and send status updates when received, awaiting information, triaged, scheduled, or closed.

FR-005 **SCHEDULING HANDOFF**

Send an accepted referral and its relevant documents to the receiving clinic's scheduling workflow without requiring duplicate patient-data entry.

FR-006 **AUDIT HISTORY**

Record access to and changes in referral status, including actor, time, previous value, new value, and reason where applicable.

QUALITY AND INTEROPERABILITY REQUIREMENTS

NFR-001 **IDENTITY MATCHING**

Match incoming patient records using the network's approved identifiers. Uncertain matches must enter a human review queue; the system must not silently merge patient records.

NFR-002 **PRIVACY AND ACCESS**

Restrict access by care relationship and role, log access to patient information, and support timely removal of access when staff roles change.

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QUALITY AND ROLLOUT REQUIREMENTS

NFR-003 **INTEROPERABILITY**

Support exchange using HL7 FHIR where both endpoints allow it. The exact FHIR release, profiles, terminology bindings, and message ownership are not yet selected.

NFR-004 **AVAILABILITY**

Provide 24-hour access for authorized users. A target availability percentage and planned-maintenance window have not been approved.

NFR-005 **PERFORMANCE**

Display a referral status update within 60 seconds after a receiving system confirms the change. The measure begins at confirmation, not at initial referral submission.

CON-001 **CLINICAL AUTHORITY**

The exchange supports workflow coordination only. Clinical urgency decisions and treatment decisions remain the responsibility of licensed clinicians.

ACCEPTANCE AND ROLLOUT

Before pilot launch, the network will verify representative referral types across at least two specialties, confirm access controls with clinical and privacy reviewers, and reconcile referral status between sending and receiving systems. Pilot scope, sample size, service levels, and clinical safety sign-off authority remain to be agreed.

Open decision

The baseline says status must be updated within 60 seconds, but does not state whether this is a 95th-percentile target, a maximum, or a target measured only when both endpoint systems are available.

ITEMS FOR REQUIREMENTS REVIEW

- Confirm supported EHR products, interface versions, and the system of record for referral status.
- Define patient identity fields and the safe handling of duplicate or uncertain matches.
- Agree the referral data-retention period, deletion workflow, and legal-hold behavior.
- Set availability, recovery, support-hours, and downtime-notification targets.
- Define clinical safety review, pilot exit criteria, and escalation ownership.